

	EMERGENCY MEDICAL SERVICES AGENCY	POLICY NUMBER	910
	A Division of the Merced County Department of Public Health		
SUBJECT: MULTIPLE CASUALTY INCIDENT (MCI) – FIELD OPERATIONS		Effective Date	07/2024
Authority: California Health and Safety Code Division 2.5 Sections 1797.220 and 1797.252 California Code of Regulations, Title 22, Division 9, Sections 100063 and 100146 FIREScope Fire Service Field Operations Guide ICS 420-1 (2022) California Code of Regulations (SEMS), Title 19, Division 2, Section 2400 2400 Government Code Section 8607 (a)		Initial Date:	10/2003
		Review Date:	07/2026
		PAGE	1 of 7

I. PURPOSE

To establish the standards for prehospital personnel in providing care during a declared Multi-Casualty Incident and to provide effective coordination and communications among incident responders, dispatch centers, area hospitals, EMS Agency Staff, Emergency Operations Center's (EOC), and local/state/federal agencies through the provision of standardized medical response policies and procedures.

This policy attempts to provide the necessary information for a rapid response, treatment and transport of victims and patients by describing the roles, relationships, and responsibilities of responding personnel, dispatch centers, and hospitals.

II. POLICY

All Multiple Casualty Incident operations within Merced County shall follow the Incident Command System and the procedures established herein. All medical resource requests shall be coordinated with the Incident Commander.

III. PROCEDURE

A. Identification of an MCI

Is determined with six (6) or more patients at a single incident or the report of 6 or more victims involved in a single incident before the arrival of public safety or EMS. The Dispatch Operations also includes the activation of an MCI response on incidents involving mass transportation vehicles (bus, train, etc.) and low visibility incidents (fog, dust) that has historically involved a multi-ambulance response.

B. MCI Alert to Disaster Control Facility (DCF)

An MCI alert to the Base Hospital, Mercy Medical Center which serves as the county's disaster control facility, should occur as soon as there is information that an MCI may exist (pre-alert). If this

Signatures on File

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STRIVING FOR EXCELLENCE

Subject:	Policy Number 910
MULTIPLE CASUALTY INCIDENT (MCI) – FIELD OPERATIONS	

occurs at the time of dispatch or while responding to the incident, the DCF will be contacted and advised of an "MCI Alert". Information should contain the location, approximate number of victims (if known), and a description of the incident should be given (i.e. Hazmat, Trauma, Medical).

C. Authority for and Confirmation of MCI

Immediately upon arrival on-scene or upon patient count update from on-scene first responders, the first arriving ALS crew or Field Supervisor shall:

1. Confirm or cancel the MCI alert with DCF.
2. Confirm location of MCI to the DCF.
3. Number of patients.

NOTE: If ten (10) or patients the EMS Dispatch Center will notify the EMS Agency on-call personnel.

D. MCI Management

Medical Group Supervisor - Once it has been determined that an MCI exists, the first arriving paramedic shall coordinate with the Incident Commander (IC) and, if assigned, assume the Medical Group Supervisor position. All additional positions and functions will be assigned as necessary to mitigate the incident and shall be consistent with the Incident Command System (ICS). The following positions fall under the Medical Group Supervisor:

- a. Triage Unit Leader.
- b. Treatment Unit Leaders.
- c. Medical Supply Coordinator.
- d. Transportation Unit Leader.

Transportation Unit Leader - responsible for coordinating all patient transportation and for completion of the Field Disposition Summary form (**Attachment C**). The following positions fall under the Transportation Unit Leader:

- a. Medical Communications Coordinator.
- b. Air Ambulance Coordinator. Landing Site Coordination shall be the responsibility of the fire agency with jurisdiction for the area of the incident.
- c. Ground Ambulance Coordinator.

E. MCI Operations

The first ALS ambulance/Supervisor on scene will, if assigned by the IC, will assume Medical Group Supervisor, Medical Communications Coordinator and Treatment Unit Leader, until resources allow for assignment to other personnel.

F. Triage

Initial Triage shall be performed utilizing the **S.T.A.R.T. method** (START Adult Triage – **Attachment A**).

1. Triage Categories:
 - a. Immediate (Red)
 - b. Delayed (Yellow)
 - c. Minor (Green)
 - d. Deceased (Black)
2. If staffing permits, re-triage shall be more precise than the initial **S.T.A.R.T. method**.

G. Treatment

1. During an MCI, ALS personnel will operate under standing orders.
2. CPR shall not be initiated unless appropriate treatment and transportation are available for the immediate treatment of all immediate and delayed patients.

H. Transportation

1. The DCF will advise the Medical Group Supervisor or designee of hospital and resource capability.
2. The Medical Group Supervisor or designee will be responsible for determining transportation mode for all patients on scene.
3. Patients are triaged utilizing the **S.T.A.R.T. method**. Trauma triage criteria does **not** decide patient destination during an MCI.
4. The Medical Group Supervisor or designee shall notify the DCF of patient destinations, categories, and counts.
5. Transportation Call-in - When a patient or patients are ready for transport, the Medical Group Supervisor, Patient Transportation Unit Leader, or designee will advise the Base Hospital/DCF of the following:
 - a. Triage Tag Number (last 4 digits)
 - b. Age
 - c. Patient Care Priority Designation (color)
 - d. Chief Complaint
 - e. Patient Destination
 - f. Transporting Ambulance Number

I. Communications

During an MCI, it is essential that all radio/phone transmissions be kept to a minimum. To ensure adequate radio/phone time, the following guidelines shall be adhered to:

1. During transport – updates with the DCF by transporting units will only be made if there is a

Subject:	Policy
MULTIPLE CASUALTY INCIDENT (MCI) – FIELD OPERATIONS	Number 910

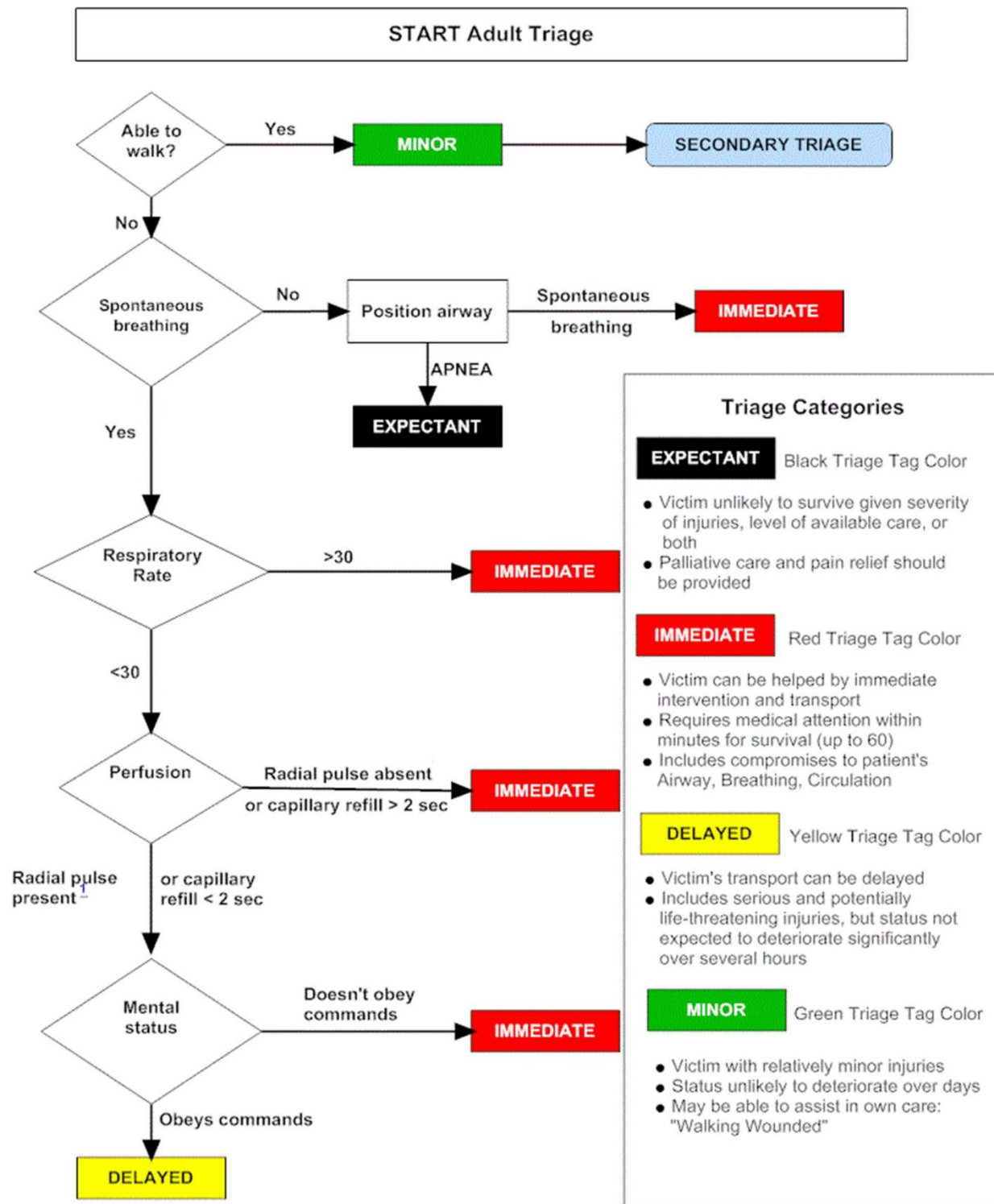
change (worsening) in the patients triage category.

2. All Other System Radio/Phone Traffic:

- a. Only that communication which is necessary for patient/personal safety or operational integrity shall be conducted during an MCI.
- b. All necessary communication shall be less than thirty (30) seconds and only contain information regarding patient(s) category, destination, and ETA.

J. Documentation

1. Triage Tags – Upon arrival at the scene, the Medical Group Supervisor or designee will distribute Fire Scope approved triage tags (**Attachment B**) to qualified triage personnel. These tags will be completed in accordance with the Merced County Patient Documentation Policy.
2. Base Hospital Routing Log — Copies of the Base Hospital Routing Log will be forwarded to the EMS Agency within 72 hours.
3. Field Disposition Summary Form – Copies of the Field Disposition Summary Form will be forwarded to the EMS Agency within 72 hours.
4. All patient contact will be documented in accordance with the **EMS Policy #540 Patient Documentation**.

ATTACHMENT A

Subject:

MULTIPLE CASUALTY INCIDENT (MCI) – FIELD OPERATIONS

Policy

Number 910

ATTACHMENT B

CALIFORNIA STATE FIRE CHIEFS' ASSOCIATION TRIAGE TAG

TRANSPORTATION RECEIPT Tag Number **4869**
E10004869

Destination _____ Via _____ Time _____
Chief Complaint _____

TRIAGE TAG E10004869
Age _____ M F
First _____ M.
Last _____
Address _____
City _____ St _____ Zip _____
Phone _____ Religious Pref _____

If Contaminated DMS-05006M • Rev 8-11
USA RADIOLGICAL AGENT BIOLOGICAL AGENT See Reverse

Blast Injury
☐ Blunt Trauma
☐ Burn
☐ C-Spine
☐ Cardiac
☐ Crushing
☐ Fracture
☐ Laceration
☐ Penetrating Injury
☐ Other _____

AFN ☐ Chief Complaint _____
Mechanism of Injury _____

V	Time	B/P	Pulse	Respiration
I				
T				
A				
L				
S				

RE-TRIAGED
1 **2** **3**
EXPECTANT 0 E10004869
IMMEDIATE 1 E10004869
DELAYED 2 E10004869
MINIMAL 3 E10004869

EVIDENCE CONTAMINATED E10004869

TRANSPORTATION RECEIPT For Use by Transportation Recorder Only DMS-05006M

Age _____ M F
First _____ M.
Last _____

☐ Tourniquet Applied Time _____
☐ Airway Management ☐ OPA ☐ NPA

GCS Tx In: E: _____ M: _____ V: _____ Time: _____
 GCS Tx Out: E: _____ M: _____ V: _____ Time: _____

Known Allergies: _____
Treatment Administered/Comments _____

D	Time	Drug Solution	Dose
R			
U			
G			
S			

INJECTOR TYPE
☐ S Salivation ☐ L Lacrimation ☐ U Urination ☐ D Defecation ☐ G G.I. Distress ☐ E Emesis ☐ M Miosis
INJECTOR TYPE _____ 1 2 3

Primary Decon ☐ 2ndary Decon ☐ **SOLUTION** _____

START triage system Initial Ribbon Triage **1 2 3**

MINOR Move the Walking Wounded
RESPIRATIONS ☐ Yes ☐ No **R**
PERFUSION ☐ - 2 Sec. ☐ + 2 Sec. **P**
MENTAL STATUS ☐ Can Do ☐ Can't Do **M**

IMMEDIATE Respirations - Over 30
IMMEDIATE Perfusion - Cap. Refill Over 2 sec. or Radial Pulse Absent
IMMEDIATE Mental Status - Unable to Follow Simple Commands
DELAYED All Others
EXPECTANT No Respirations After Head Tilt

EXPECTANT 0 Pulseless/ Non-Breathing
IMMEDIATE 1 Life Threatening Injury
DELAYED 2 Serious Non Life Threatening
MINIMAL 3 Walking Wounded

EVIDENCE CONTAMINATED

MULTIPLE CASUALTY INCIDENT (MCI) – FIELD OPERATIONS

Number 910

Emergency Medical Services Agency

Field Disposition Summary Form - MCI Transportation Log	
Run#:	Location:
Date:	MCI Name:
Patient Count	
Patient Category	Totals
MINOR (Walking Wounded)	
DELAYED (Serious but Non-Life Threatening)	
IMMEDIATE (Life Threatening Injury)	
11-44 / MORGUE (Pulseless/Non Breathing)	
Total Number of Patients	

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Page 7 of 7